

POWERED BY **ZAIZENS** · AN INTEGRATED HMS

PREMIUM · PROFESSIONAL EDITION

Emergency / A&E Department

Complete staff guide — rapid registration, triage, concurrent care, parallel orders, disposition, medico-legal cases, and retrospective billing.

PRODUCT

ZAIZENS Integrated HMS

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Emergency User Manual

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Introduction

Welcome to the **ZAIZENS Integrated HMS Emergency User Manual**. This document covers the full Accident & Emergency (A&E) workflow — designed for critical patients where **prepayment is bypassed** and care begins immediately on a **credit tab**.

Critical rule: For emergency patients, select **Emergency / Waiver** at registration (or use A&E Quick Registration). This activates prepayment bypass and routes the case to the Emergency board — not standard OPD cashier-first flow.

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1. Emergency Overview & Roles

Emergency / A&E manages critically ill or injured patients with concurrent nursing and medical assessment. Orders and medications accumulate on an open **credit tab** until disposition and cashier settlement.



Front Desk

Rapid registration, Emergency/Waiver, A&E quick reg — bypass prepayment



Nurse / Nursing Aid

Immediate vitals, acuity assignment, bed assignment, continuous monitoring



Doctor

Immediate assessment, SOAP, parallel orders (lab/rad/pharm/blood), disposition



Lab / Radiology

Urgent investigations — STAT priority; results to chart immediately



Pharmacy

Stat doses, emergency medications on credit tab



Cashier

Retrospective bill presentation and settlement after stabilization

Key URLs

Screen	Path	Roles
Emergency board	/emergency	All A&E staff
ER visit chart	/emergency/visit/:id	Doctor, Nurse
ER KPI dashboard	/emergency/kpi	Supervisors, Admin
Doctor ER alerts	/doctor/er-inbox	Doctor
OPD queue (switch)	/opd-queue	Front Desk — Emergency/Waiver
Ward board / ADT	/wards	Admit from ER
ER cashier tab	/cashier (A&E filter)	Cashier

Separate from OPD: Emergency patients on the ER board do not appear on the standard OPD queue when assigned to an ER doctor.

emergency.read

emergency.write

2. Four-Phase ER Workflow

The revamped Emergency module follows **four clinical phases** on each visit chart (`/emergency/visit/:id`):

PHASE 1 — ARRIVAL · QUICK REGISTRATION · TRIAGE

Patient arrives → quick reg (existing or new/unknown) → acuity level, vitals, flags, chief complaint, bed assignment.

PHASE 2 — PARALLEL ORDERS · SOAP CONSULTATION

Doctor assessment concurrent with nursing. Place STAT lab, radiology, pharmacy, blood bank, or procedure orders on credit tab.

PHASE 3 — DISPOSITION

Once stabilized: discharge home, short stay (SSU), admit IPD, direct to OT, transfer out, deceased, or left without being seen (LWBS).

PHASE 4 — CHARGE CAPTURE · ER CLOSURE · BILL SETTLEMENT

Real-time charges on open tab → confirm disposition → cashier retrospective settlement → visit closed.

Arrival → Triage (Acuity + Vitals) → Doctor SOAP + Orders (concurrent)
→ Disposition → Charges settled at Cashier → [Admit to IPD if required]

3. Eight-Step Emergency Process

Role-based steps aligned with workflow guides and live ER board:

1 Front Desk — Rapid registration. Tick **Emergency / Waiver**. No prepayment — billing exception created. `opd.write`

2 System — Auto-routed to Emergency/A&E. Status → Waiting Doctor. ER widgets update on dashboards.

3 Nurse — Record vitals immediately: BP, Temp, SpO₂, Pulse, GCS, pain score. Concurrent with doctor. `nursing.write`

4 Doctor — Immediate assessment and treatment. Rapid orders (IV, bloods, imaging, meds) on credit tab. `consult.write`

5 Lab / Radiology — Process urgent investigations. Upload results immediately to chart.

6 Pharmacist — Dispense emergency medications. Costs on credit tab.

7 Doctor — Once stabilized: admit to IPD, or discharge with documented disposition.

8 Cashier — Present retrospective bill. Settle all emergency charges. Issue receipt.

4. Acuity & Triage Levels

Triage assigns an **acuity level** (L0–L5) with SLA targets for door-to-doctor time. Patients appear in color-coded lanes on the Emergency board.

Level	Label	SLA	Typical presentation
L0	Awaiting triage	—	Just registered — assign acuity
L1	Resuscitation	Immediate	Cardiac arrest, major trauma
L2	Emergent	< 10 min	Chest pain, stroke symptoms
L3	Urgent	< 30 min	Moderate injury, fever
L4	Less urgent	< 60 min	Minor laceration, mild symptoms
L5	Non-urgent	< 120 min	Primary care suitable

Triage flags

Mark clinical flags during triage: Trauma, Cardiac, Stroke, Pediatric, Psychiatric, Obstetric — for routing and KPI reporting.

ER bays

- **Resuscitation** — critical care bay
- **Holding** — pending bed or disposition
- **Observation** — short-term monitoring
- **Short Stay Unit (SSU)** — < 24h observation pathway

5. Emergency Board

The live **Emergency Department** board at `/emergency` shows:

- **KPI strip** — active cases, awaiting triage, critical alerts, door→doctor median, open MLC cases
- **Credit tabs summary** — total FCFA across open emergency visits
- **Acuity lanes** — patients grouped by L0–L5 with wait times and SLA highlighting
- **My ER patients** — doctor-assigned cases (not on OPD queue)

Opening a visit

Click any lane card to open `/emergency/visit/:id` — full four-phase chart with triage, orders, disposition, and charges.

Top navigation **ER** link (red) opens the Emergency board directly from any screen for urgent access.

6. Quick Registration

A&E Quick Registration modal

- 1 Click **New emergency patient** on Emergency board.
- 2 **Phase 1 — Arrival:** choose existing patient or new/unknown.
- 3 For new: enter name, gender, phone (or Unknown), chief complaint.
- 4 Assign doctor (or any available).
- 5 Submit **Register & start triage** — bypasses cashier prepayment.

From OPD queue

When adding an OPD visit, tick **Emergency** — system prompts switch to A&E Quick Registration with waiver message.

Unknown / walk-in

Use **New / Unknown** path for unidentified patients; complete demographics when available during care.

7. Phase 1 — Triage

On the ER visit chart, complete **Phase 1 · Triage**:

- 1 Set **Acuity Level** (L1–L5).
- 2 Enter vitals: BP sys/dia, pulse, SpO₂, temp, respiratory rate, GCS, pain 0–10.
- 3 Set triage flags (trauma, cardiac, stroke, etc.).
- 4 Record **Chief complaint** (clinical).
- 5 **Assign bed** in resuscitation, holding, observation, or SSU bay.
- 6 Save triage — vitals sync to nursing chart for OPD consultation rules.

Update triage as condition changes. Critical results trigger alerts on the board.

8. Phase 2 — SOAP & Parallel Orders

Consultation workflow

- 1

Mark Doctor Seen — starts door-to-doctor KPI timer.
- 2

Create consultation — SOAP note required before prescriptions and some clinical docs.
- 3

Concurrent nursing continues vitals and monitoring.
- 4

Use **Phase 2 · Parallel Orders** for urgent requests.

Order types

Type	Examples	Priority
Laboratory	CBC, cross-match, blood gas	STAT / Urgent / Routine
Radiology	Chest X-ray, CT head	STAT
Pharmacy	Stat dose medications	STAT
Blood bank	Transfusion products	STAT
Procedure	Suturing, chest drain	Urgent

Orders show charge preview on credit tab. Lab/radiology alert department inboxes — use **New Order** in Phase 2 for tests (Phase 4 charges alone do not authorize tests).

9. Phase 3 — Disposition

When clinically stable, choose a **disposition pathway**:

Pathway	When to use
Discharge home	Stabilized — settle bill at cashier; take-home meds & return precautions
Short Stay (SSU)	< 24h observation before final disposition
Admit to IPD	Requires hospitalization — ER charges transfer to IPD bill
Direct to OT	Emergency surgery — document procedure
Transfer out	Refer to another facility
Deceased	Record death — opens death registry
LWBS	Left without being seen — waives pending billing per policy

Enter clinical summary and confirm disposition. Visit timeline records arrived, doctor first seen, and door→doctor minutes.

10. Phase 4 — Charges & Settlement

Phase 4 • Charges on the ER visit chart:

- View all line items on open credit tab
- Add manual charges (catalog, pharmacy search, miscellaneous)
- Mark items settled as cashier processes payment
- **Total open tab** displayed until closure

After disposition, visit moves toward **clinical discharge** state — chart becomes read-only for clinical edits; awaiting financial settlement.

EMG settlement tickets

Cashier workspace shows emergency patients with unpaid charges. Filter A&E badge cases for pending settlement.

11. Medico-Legal Cases (MLC)

For assault, RTA, poisoning, burns, sexual assault, and other notifiable cases:

- 1 Flag visit as **MLC** on ER chart.
- 2 Complete MLC form: case type, incident time/place, brought by, police details.
- 3 Document examination, injuries, provisional diagnosis.
- 4 **Notify police** when required.
- 5 **Sign & Lock** — record becomes read-only; print triplicate copies.

Once MLC is locked, it cannot be edited — only viewed and reprinted. Open MLC count appears on Emergency board KPIs.

12. ER → IPD Admission

When disposition is **Admit to IPD**:

- 1 Doctor confirms admitting department and diagnosis.
- 2 System creates IPD admission request.
- 3 ER charges may **transfer to IPD running bill** when configured.
- 4 ADT assigns ward and bed on ward board.
- 5 Follow IPD User Manual for deposit, nursing, and discharge workflows.

Maternity emergencies may link to labor ward and neonatal pathways with pay-later deposit rules.

13. Retrospective Billing

After clinical care

- 1 Patient or family directed to cashier when medically appropriate.
- 2 Cashier filters **Emergency / A&E** pending tickets.
- 3 Review full credit tab: consultations, orders, medications, procedures.
- 4 Collect payment or apply insurance co-pay.
- 5 Issue receipt — mark EMG settlement complete.

Emergency flow intentionally defers payment until after stabilization — unlike standard OPD consultation-first payment.

14. KPI Dashboard

ER KPI dashboard at `/emergency/kpi` tracks:

- Door-to-doctor time (median / SLA breaches)
- Left without being seen (LWBS) rate
- ER → IPD conversion rate
- Critical alert counts
- MLC cases open
- Credit tab totals across active visits

Use for shift handover, quality improvement, and hospital management reporting.

15. Doctor ER Alerts

Doctors receive real-time alerts at `/doctor/er-inbox` for:

- Critical lab/radiology results
- Stat order completions
- Medication administration notifications
- Assigned ER patients requiring review

Red **ER alerts** strip appears on clinical pages when pending items exist.

16. Screen Reference (UI)

Illustrative panels matching live Emergency React screens.

Emergency Board — /emergency

Live · Emergency / A&E

Active: 8 Awaiting triage: 2 Critical: 1

L1 · Resuscitation · J. Doe · 3 min · MLC

L2 · Emergent · P. Kam · 8 min · Chest pain

L3 · Urgent · A. Mbu · 22 min

ER Visit Chart — Phase panels

PHASE 1 · TRIAGE ✓

PHASE 2 · ORDERS (3 ACTIVE)

PHASE 3 · DISPOSITION — CHOOSE PATHWAY

PHASE 4 · OPEN TAB: 45,000 FCFA

Quick Registration

Quick registration · Phase 1 — Arrival

☒ Existing patient ☐ New / Unknown

Chief complaint: *Severe abdominal pain*

Register & start triage

Live screenshots: Open `/emergency` while signed in; use Print → Save as PDF for site-specific training packs.

17. Troubleshooting

Issue	Solution
Patient sent to OPD cashier first	Re-register with Emergency/Waiver or use A&E Quick Reg
Patient not on ER board	Confirm emergency flag; check assigned doctor ER queue not OPD
Cannot place orders	Create SOAP consultation first; verify visit not closed
Lab not processing	Use Phase 2 New Order — charges alone do not authorize tests
Cannot edit chart	Visit closed (clinical discharge) — financial settlement pending or complete
MLC cannot edit	MLC is locked after Sign & Lock — reprint only
Admit to IPD blocked	Complete disposition pathway; check bed availability on ward board
LWBS still showing charges	LWBS disposition waives billing per policy — confirm pathway confirmed

18. Glossary

Term	Definition
A&E	Accident & Emergency — same as Emergency Department
Credit tab	Running bill for ER orders before cashier settlement
Acuity	Triage severity level L1 (highest) to L5 (lowest)
STAT	Immediate priority order
SOAP	Subjective, Objective, Assessment, Plan — consultation note
Disposition	Final clinical pathway (discharge, admit, transfer, etc.)
MLC	Medico-Legal Case — assault, RTA, notifiable injury documentation
LWBS	Left Without Being Seen
SSU	Short Stay Unit — brief observation
Door-to-doctor	Time from arrival to first doctor contact
ER waiver	Prepayment bypass for emergency registration